Palbociclib with endocrine therapy

Take palbociclib for 21 days, then have 7 days without palbociclib. The aromatase-inhibitor tablet continues every day, or fulvestrant injections follow their separate schedule. The week off applies to palbociclib, not automatically to the hormone treatment.

Oral CDK4/6 inhibitor + hormone therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Blood counts

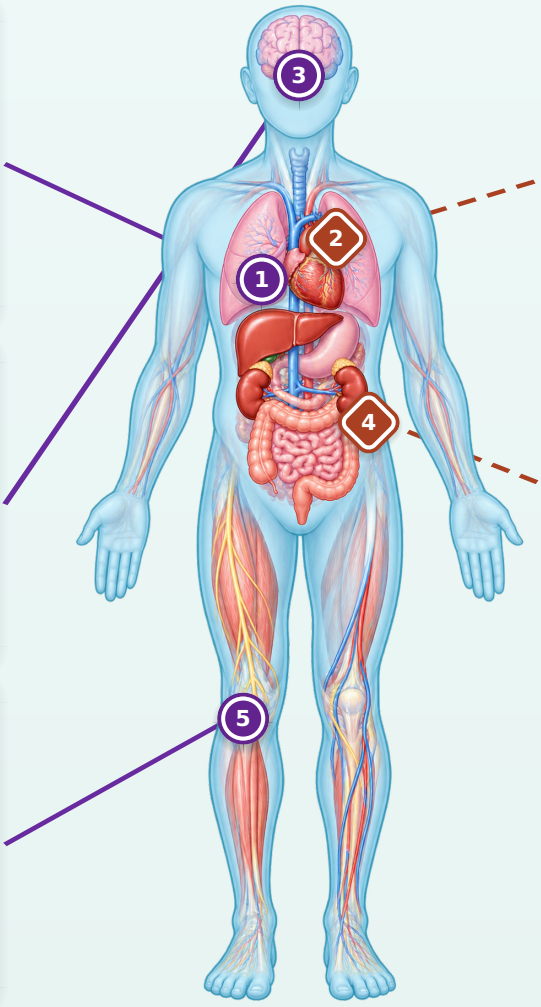
Palbociclib

Counts can fall without symptoms. Keep FBC appointments and report infection symptoms.

2 Lungs

Palbociclib

New cough or breathlessness needs prompt assessment for infection or uncommon lung inflammation.



3 Mouth

Palbociclib

Report painful sores or difficulty eating/drinking.

4 Energy

Palbociclib

Report fatigue that limits activity or is worsening; blood counts and other causes may need review.

5 Joint / bone symptoms

Endocrine partner

AI and ovarian suppression may affect joints and bone; agree the partner-specific bone plan.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the treatment team promptly if you feel feverish or unwell, have shaking chills or infection symptoms, or develop new cough or breathlessness. Follow the local emergency temperature instructions; do not wait for a planned blood test or for the 38.5°C value used in one clinician dose-adjustment table. Severe breathing difficulty needs emergency care. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

YOUR PRACTICAL TREATMENT PLAN

Know the next step.

Taking treatment

Take palbociclib for 21 days, then have 7 days without palbociclib. The aromatase-inhibitor tablet continues every day, or fulvestrant injections follow their separate schedule. The week off applies to palbociclib, not automatically to the hormone treatment.

Check whether the pharmacy supplied capsules or tablets: capsules are taken with food; tablets can be taken with or without food. Swallow whole at a consistent time. Avoid grapefruit. If you miss or vomit a dose, do not take an extra dose; take the next scheduled dose. Do not restart after a clinical hold until the treating team confirms.

Checks and follow-up

Before the first dose: FBC, kidney and liver bloods; review other medicines, pregnancy possibility, receptor results, menopausal status and the exact endocrine partner. Confirm ovarian suppression when relevant. FBC on Day 1 and Day 15 of cycles 1–2, then Day 1 from cycle 3. Kidney/liver bloods before each cycle. If counts are low, extra blood tests and a pause may be needed. Only after no worse than grade 1/2 neutropenia in the first 6 cycles may FBC frequency reduce to every 3 months before a cycle, with additional tests when clinically indicated.

The whole-treatment map shows the checks for this exact treatment setting.

Support between reviews

Nausea support

Palbociclib is classified as oral minimal–low emetic risk. Routine premedication is not usually needed; discuss rescue medication if nausea occurs. A generic intensive chemotherapy antiemetic schedule is not automatically added.

Low counts and infection

Blood-count monitoring, dose interruption and dose reduction are central to support. Growth-factor use is an individual clinical decision, not an automatic routine injection on this card.

Hormone-related symptoms

Discuss hot flushes, joint symptoms, vaginal symptoms, sleep and bone health according to the actual endocrine partner. Support can be adjusted without silently changing either cancer medicine.

Mouth, bowel and energy symptoms

Report mouth soreness, troublesome nausea/diarrhoea or fatigue so supportive treatment and hydration can be reviewed. New breathing symptoms must not be dismissed as expected fatigue.

Oncology daytime: Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital’s urgent-care plan. Emergency: 112 / 999.

Your complete treatment reference

https://sactcheck.com/patient/00414/

Public information only. No personal information is encoded.